

A Lifelong Hard Gainer's Road to Recovery & Hypertrophy

The Evolution of an Investigation

An Investigative Monograph

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This publication documents a longitudinal self-case investigation conducted by the author. It is intended for educational, investigative, and discussion purposes. It is not medical advice, diagnosis, or treatment.

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Series Statement

This publication is one volume in the *A Lifelong Hard Gainer's Road to Recovery & Hypertrophy* series.

Edition Statement

This edition follows the reasoning itself, tracing not only what was discovered but how each explanation gave way to the next.

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Chapter One: Introduction

Something was wrong with the left side of my body, and I could not train my way out of it.

That sentence sounds simple. It took years to be able to say it plainly. For a long time, the truth was buried under a much more comfortable story — the story that a particular muscle was injured, that it would heal, and that when it healed, everything downstream of it would heal too. That story was not stupid. It was not lazy thinking. It was the correct application of ordinary experience to a problem that, it would turn out, was not ordinary at all.

This book is not really about my left leg. My left leg is where the evidence happened to live. It is the setting, not the subject. The subject is something closer to a question every person who has ever tried to solve a hard problem eventually runs into: how do you know when a good explanation has stopped being a true one?

I did not set out to write an investigation. I set out to fix a shoulder, and then a leg, and I expected the fixing to look like it always had — identify the tight thing, release the tight thing, strengthen around it, move on. For most of my life, that approach had worked well enough. I had always been strong for my size, always physically capable in ways that made “just train through it” a reasonable default. So when a new restriction showed up on my left side, I did what I had always done. I treated it like a known problem with a known shape.

It did not behave like a known problem. It behaved like something that kept generating new symptoms every time I thought I had found its edge.

By the time I recognized this — really recognized it, not just noticed it in passing — I had already spent years assuming that persistence and conventional effort would eventually be enough. They were not. A serious medical event took the choice out of my hands entirely for half a decade, and when I finally returned to training, the left side of my body still would not do what the right side did without thinking about it. Conventional strengthening, applied patiently and correctly, produced almost no change. That failure is where this book actually begins — not with an exercise, and not with an injury, but with the moment the obvious explanation stopped being sufficient and I had to admit I did not understand what I was looking at.

What follows is an account of how that understanding was built, one wrong answer at a time. I want to be honest about that phrase — *wrong answer* — because most of the explanations in this book were not wrong in the sense of being foolish or careless. They were wrong the way a first sketch is wrong: recognizably related to the final shape, useful for getting started, and still not the thing itself. Several of them remained partially true even after being replaced. None of them were abandoned lightly. Each one was believed, tested, and only set aside when the evidence stopped cooperating with it.

That is the shape of the book you are about to read. Not: here is what happened. Instead: here is what I believed, here is why it made sense to believe it, and here is what eventually made it impossible to keep believing.

The left side of my body is still, as I write this, not fully resolved. That is not a spoiler I regret giving away. An investigation that ends with a tidy, permanent answer would actually be a less honest book than the one I am able to write. What I can offer instead is a record of how belief actually changes when you refuse to stop looking — which, if it works the way I think it does, will be useful to you regardless of whether you have ever had a single problem with your adductor, your ankle, or anything else on this particular list.

Chapter Two: Why This Investigation Was Different

The obvious move, when training stops working, is to train harder.

I had done that before, more than once, for smaller problems, and it had usually been the right call. More volume, more consistency, more patience — most setbacks in a training life respond to some combination of those three things eventually. So when I returned to lower-body training after a long, medically forced absence and found the left side unwilling to cooperate, the first instinct was not to investigate anything. It was to keep training and assume the gap would close on its own, the way gaps like it always had.

It did not close. Weeks became a couple of months, and the left side of my body was, if anything, more obviously behind than when I had started. That is a specific and uncomfortable kind of evidence — not pain, not injury, just a stubborn refusal to adapt the way the right side was adapting under identical effort. And it forced a question I had never had to take seriously before, because I had never needed to: what if the problem isn't how hard I'm training. What if the problem is that I don't understand what's actually wrong.

That question is where the investigation actually starts, more than any single symptom or injury. Everything before it was training. Everything after it was something else.

I want to be honest about how unnatural this shift felt at first. Deciding to stop asking *how can I get stronger* and start asking *why isn't normal strengthening producing normal function* is a strange thing to do to your own training program. It slows you down. It replaces a familiar kind of effort — push harder, add weight, grind through it — with a much less satisfying kind: watch closely, write things down, resist the urge to conclude anything before you actually have a reason to. For someone whose entire relationship with his body had been built on effort and persistence, this felt, for a while, like giving up on effort altogether. It wasn't. It was effort redirected at a different target. But it took time to trust that the redirection was worth it.

Once I committed to it, a handful of rules emerged — not all at once, and not because I sat down and designed them, but because they kept proving necessary in practice, the way rules of thumb usually do. Don't assume; observe. Separate what you actually saw from what you think it means. Write the observation down before you write the explanation down, because once the explanation exists, it has a way of quietly editing the memory of what you originally noticed. Compare what's happening now against whatever normal function looks like, whenever a comparison is available. And — this one mattered more than it sounds like it should — retain uncertainty for as long as the evidence allows you to,

instead of resolving it early just because an unresolved question is uncomfortable to sit with.

That last rule is the one most training programs actively work against. Programs want commitment. They want you to pick an approach and follow it. An investigation wants almost the opposite: a willingness to keep an explanation on probation indefinitely, using it while continuing to test whether it deserves to be believed. Those are different mental postures, and switching between them is harder than it sounds. I did not always manage it cleanly. There are places later in this book where I clearly wanted an explanation to be finished before it actually was, and had to walk that wanting back.

The decision to document everything — not just the successes, but the false starts, the abandoned hypotheses, the exercises that turned out to test nothing useful — came out of a specific frustration. Early on, I noticed that my own memory was doing exactly the thing I was trying to guard against: quietly rewriting earlier confusion into something that sounded, in hindsight, more confident than it had actually been at the time. If I only remembered the discoveries and let the dead ends blur together, I was going to end up with a story that flattered my own reasoning rather than one that accurately showed how the reasoning had actually developed. Writing things down as they happened — including the things that turned out to be wrong — was the only defense against that. It meant a later version of me could look back at an earlier explanation and see honestly why it had made sense at the time, rather than either pretending it had always been obviously incomplete or pretending it had never been believed at all.

There is a particular kind of intellectual honesty required to preserve a wrong idea instead of quietly deleting it once you know better, and I don't think I fully appreciated how much I would need that honesty until I was several abandoned explanations deep and starting to notice a pattern in how they failed. Almost none of them were disproven outright. Almost all of them ran out of room instead — kept being technically true while gradually becoming less and less sufficient to explain what I was actually observing. You cannot see that pattern if you only keep the final answer. You can only see it if you keep the whole trail.

So that became the discipline, stated as plainly as I can state it: observe before you explain, compare against whatever normal you can find, experiment deliberately rather than randomly, and when the working explanation eventually breaks — because it eventually always did — resist the urge to throw out everything that came before it. Keep what persists. Discard only what the evidence actually contradicts. Fold the rest into a slightly larger, slightly more honest explanation, and go looking for what that new explanation still can't account for.

Repeat. That, more than any single finding in the chapters ahead, is the actual method of this book.

Chapter Three: The First Wrong Answer

The injury itself was almost boring, which is part of why it was so easy to believe it explained everything that came after.

It happened during ordinary resistance training, on an adductor machine, doing nothing unusual. There was an acute injury, real pain that lasted for months, and a lingering tightness and weakness that did not fully resolve even once the acute phase had passed. Certain movements were affected in a specific, repeatable way — anything asking for deep hip flexion on the left side, presses and lunges especially, met a restriction that had not been there before.

If you have ever hurt something the way I hurt that adductor, you already know how the explanation writes itself. An injury happened. An injury leaves a mark. The mark explains the restriction. The restriction explains the difficulty. It is a clean, closed loop, and for a while, the evidence seemed to agree with it. Knee-to-chest movement stayed limited on that side. Certain exercises reliably produced a pinching sensation in roughly the same place. The story held together well enough that I did not think of it as a hypothesis at all. It was simply what had happened to me.

I want to dwell on why that explanation was reasonable, because the temptation in hindsight is to make it look naive. It was not naive. It was the correct explanation for a large fraction of what I was observing. The adductor genuinely had been injured. It genuinely remained tight and weak for a long time afterward. Later, much later, certain loaded movements — a Bulgarian split squat, a walking lunge under fatigue — would still occasionally produce that same pinch, months and then years after the original event. An explanation that keeps producing correct predictions, even years later, is not a bad explanation. It is simply an incomplete one, and incompleteness is very hard to see from the inside.

The first real crack in it did not come from the adductor at all. It came from paying closer attention to a much simpler movement: walking lunges, done for reasons that had nothing to do with testing a hypothesis about injury history.

What the walking lunges kept showing, if I was honest about the order of events, was that the tightening did not always start where I expected it to. Sometimes the glute and hip on the left side would tighten up first, under load, and only afterward would the familiar adductor sensation follow — as though the adductor were reacting to something happening somewhere else in the chain, rather than initiating the problem itself. That is a small observation. It is also the kind of observation that, once you actually let yourself notice it instead of explaining it away, is very difficult to un-notice.

This is the moment I want to be precise about, because it is the real turning point of the chapter, and it is easy to describe it wrong. The adductor explanation was not disproven. Nothing in the evidence said the injury hadn't happened, or that it didn't matter, or that the tightness wasn't real. What the evidence said, cumulatively and a little uncomfortably, was narrower and more specific: the adductor could no longer be trusted as the *initiating* structure. It might be downstream of something else. It might be one participant in a larger pattern rather than the author of that pattern. The dynamic, unilateral loading of a simple walking lunge was exposing something that static, controlled testing of the adductor alone had never revealed — that the restriction I had been treating as local was capable of showing up in places the original injury had no obvious business reaching.

Once that door was open, it did not close again. Over the following months, the list of things the adductor explanation could not account for kept growing: restrictions in push-off, in downward pressure through the foot, in knee and thigh stabilization, in the trunk's willingness to accept load on that side. None of these were adductor problems in any anatomical sense. All of them were happening on the same side of the same body, in the same general pattern of dysfunction, in a person whose adductor had, by any reasonable clinical standard, mostly healed.

I did not, at the time, throw the adductor explanation away. That would have been its own kind of mistake — replacing one oversimplification with another, just in the opposite direction. What actually happened was slower and less satisfying to describe: the adductor injury stopped being the explanation and started being *a* contributing history, one piece of a larger and still-unmapped picture. It remained relevant. It was simply no longer sufficient, and the difference between those two words — relevant and sufficient — turned out to be the difference between a story that felt finished and an investigation that was only just getting started.

That distinction is going to matter again. It is going to matter almost every time this book reaches a new discovery, because the same pattern repeats with almost uncanny consistency: an explanation arrives, it is genuinely useful, it survives real testing for a while, and then — not by being refuted outright, but by quietly running out of things it can explain — it reveals its own edges. The adductor was simply the first place I watched that happen. It would not be the last.

Chapter Four: The Shape of an Incomplete Answer

Somewhere in the second year of this, I stopped being surprised by the specific content of a discovery and started being surprised by its shape.

The pattern was not obvious the first time it happened. It was barely visible the second time. By the fourth or fifth time, it had become impossible not to see, and once I saw it, I could not stop seeing it everywhere I looked — in the way a word starts sounding strange once you've stared at it too long.

Here is the shape. Something that had been unreachable becomes reachable. A structural release frees up a joint that had been locked for years. A muscle that had gone quiet finally responds to a cue. A region of the foot that had refused to bear weight normally starts, tentatively, to do so. Each of these moments felt enormous when it happened — and they were not nothing. They were real, repeatable, physically verifiable changes. But every one of them was followed, weeks or months later, by the same disappointing discovery: the improvement had not actually finished the job it appeared to be finishing.

The release would restore mobility, and mobility would feel like the answer, right up until it became clear that being able to move through a range and being able to do anything useful with that range were two different achievements — and that the second one hadn't arrived just because the first one had. A muscle would come back online, genuinely, verifiably, and I

would treat that as evidence the surrounding system was repaired, only to find that one recovered muscle in an otherwise uncoordinated chain behaves less like a repaired system and more like a single working light switch in a house where the wiring is still bad. The foot would improve — real, measurable improvement in how it contacted the ground — and push-off would still not work, because contact and propulsion turned out to be different problems wearing the same shoe.

I want to slow down on that last example, because it's the clearest version of the pattern and it's worth sitting inside it rather than rushing past it. Watching the arch of my own foot begin, for the first time in years, to genuinely participate when I stood on it — actively, not just structurally sitting there — felt like it should have been close to the end of the story. It wasn't close to the end. It was one working component in a base that still, as a whole, couldn't grab the ground the way the other side could. The improvement was completely real. It was also, on its own, almost beside the point.

By the time I recognized this as a pattern rather than a coincidence, I had already lived through it three or four times without naming it. That is, I think, the honest way these things actually happen. You do not walk into a pattern and identify it on sight. You walk into it, get fooled by it, walk into it again, get fooled again — a little less thoroughly the second time — and only gradually does the fooling stop working. The moment of recognition, when it finally arrived, did not feel like a discovery about my body at all. It felt like a discovery about the *kind of mistake I kept making*. Every one of these individual improvements was relevant. None of them, alone, was sufficient. And I had apparently needed to relearn that same lesson, in a new location, every single time, before I was willing to generalize it.

Once the pattern was named, it changed how I looked at everything still in front of me. I stopped asking whether a new improvement was real — by this point, I mostly trusted that it was — and started asking a colder, more useful question instead: real, compared to what? Real enough to explain the whole problem, or real enough to explain one more piece of it? That question does not make discoveries feel smaller. If anything, it makes you more careful about which discoveries you're willing to call finished, and more alert to the parts of the picture a given breakthrough conveniently leaves out.

I did not yet know what would eventually tie all these disconnected, partial improvements together. I only knew, by this point, that whatever it was would have to explain something the individual pieces couldn't: why release, and muscle, and foot could each genuinely improve on their own, and the whole left side of my body could still, collectively, refuse to function like the right.

That answer was close. I did not know how close.

Chapter Five: The Explanation That Kept Explaining

It started with an exercise I regarded as routine — a very light, low-load leg press, the kind of thing you do to warm a joint up rather than to train anything seriously.

That is worth pausing on, because it is not the kind of detail that fits the story we usually tell about breakthroughs. There was no hypothesis going into that set. I was not testing anything. I was priming, using a light horizontal press the way you'd use any other low-stakes warm-up movement, and somewhere in the middle of an unremarkable set, something small and specific caught my attention: a distinction in how the two sides of my body were organizing themselves at the ankle, a difference too subtle to have shown up under any heavier, more demanding load. The weight had been so low that it let mechanics surface which every previous, more serious attempt at this problem had been too loaded to reveal.

I noticed it. I did not, at that moment, understand it. That gap — between noticing something and understanding it — turned out to matter more than almost any other moment in this book, because what I did with that gap was the thing that made the discovery possible at all. I did not dismiss the observation for being too small or too strange to fit anything I already believed. I carried it with me.

A few days later, walking, it happened again — a related sensation, this time completely outside a gym, completely outside any deliberate testing. Two data points from two unrelated contexts are a different kind of evidence than one. They suggested a pattern rather than a fluke, and a pattern, unlike a fluke, is something you can go looking for on purpose.

So I did. I went home and mapped it — deliberately, methodically, comparing one side of my body against the other with an attentiveness I had not brought to this specific question before. What I found was that the muscular actions controlling my foot at the ankle, in each of four directions, corresponded to distinctly different sensations of engagement running up through the hip and trunk on that same side. My right ankle could organize itself through essentially the full range of that control without any conscious thought — a kind of full, automatic bracing I had simply never had reason to notice, because it had never been missing. My left ankle could not do this. Not automatically. Not yet.

I want to be careful here about how I describe what this discovery was, because it would be easy — and wrong — to say that the ankle turned out to be the answer. It was not the answer. It was the first genuinely new category of information the investigation had produced in a long time. Every previous discovery had been asking, in one form or another, a proximal question: what was happening at the hip, the glute, the trunk. This was the first time the evidence pointed convincingly toward the *base* of the chain as an active, controllable, currently under-functioning participant — not just a passive foundation the rest of the system stood on, but a structure with its own job to do, a job the left side was not yet doing on its own.

I applied it a few days later, deliberately, during an ordinary lower-body training session. I want to resist making that session sound more dramatic than it actually felt at the time, because in the moment, it mostly just felt like paying very close attention to something I usually ignored. But the results were immediate and, by that point in the investigation, unusually consistent across nearly everything I did that day. A neutral leg press, performed slowly and deliberately, produced a sensation of load acceptance running further through

the trunk than I remembered it ever running before. A split squat produced a clear, continuous line of force from heel to hip that had previously felt disconnected in the middle. Even a standing calf raise — an exercise I had done thousands of times without thinking about it — produced a whole-leg response, a kind of trembling, engaged participation from ankle to hip that had simply never been there.

None of these individual results would have meant much on their own. Any one of them, in isolation, would have been just another entry in the long list of promising things that later turned out to be partial. What made this different — what makes me still willing, months later, to call it the discovery that changed everything — was that it was the first finding capable of explaining *multiple* of the chain's separate, previously unconnected deficits at once. It did not explain everything. It never claimed to. The proximal restrictions I had identified earlier remained exactly where they had always been, unresolved and untouched by anything happening at the ankle. But for the specific, longstanding, deeply frustrating inability to brace at the base of my body and push down into the ground normally — a problem I had lived with for so long I had nearly stopped treating it as a problem — this was the first explanation with enough reach to actually account for it.

I had spent a long time, by this point, being suspicious of exactly this feeling. Every previous breakthrough had also felt, in its first days, like it might finally be the whole answer. I had learned, the hard way, not to trust that feeling uncritically. So I want to say plainly what made this one different, rather than let the reader assume I simply got lucky and stopped being careful: this discovery was different because of what it explained, not because of how it felt. It felt exactly like all the others had felt. What set it apart was that it kept explaining more, for longer, without running into an obvious edge the way every previous discovery eventually had.

That, as it turned out, was the beginning of a different kind of question — not *what else is wrong*, which had driven nearly everything up to this point, but *what does all of this actually add up to*. That question is where the next chapter lives.

Chapter Six: The Biggest Lesson

If you have read this book in order, you already know what I am about to say, because you have watched it happen four times before I say it.

Every real breakthrough in this investigation initially looked like the answer. Every single one. The adductor looked like the answer. The proximal releases looked like the answer. Individual muscle re-engagement looked like the answer. The ankle discovery, dramatic and genuinely important as it was, also looked like the answer, for at least a few days, before its actual boundaries became clear.

None of them were lying to me, exactly. That is the part of this lesson that took the longest to actually absorb, rather than just intellectually accept. I kept expecting, somewhere in this process, to catch myself being fooled by something false — a bad observation, a wrong measurement, a hopeful misreading of an ambiguous result. That happened sometimes, but

it was never the main story. The main story, over and over, was that a real, true, well-supported finding simply did not extend as far as its own initial success suggested it might. The evidence wasn't wrong. My estimate of what the evidence was capable of explaining was wrong, almost every time, in the same optimistic direction.

I think there is a specific reason this keeps happening, and it is worth naming plainly, because I do not think it is unique to investigations like this one. When an intervention works — when a release genuinely restores motion, when a cue genuinely improves a lift, when a discovery genuinely changes how your body feels for the first time in years — the success itself generates a kind of narrative pressure toward closure. It feels finished. The relief of the improvement and the belief that the problem is solved arrive together, as though they were the same feeling, and pulling them apart requires a specific, unnatural discipline: continuing to ask what the improvement doesn't explain, at the exact moment when everything in you wants to stop asking questions and just enjoy having solved the problem.

I did not always have that discipline. There are explanations in earlier chapters of this book that I held onto slightly longer than the evidence deserved, because letting go of a good feeling is harder than logic alone accounts for. What saved the investigation, when it was saved, was not that I never made this mistake. It was that I had built a habit, by that point, of writing down what each new explanation could not yet account for, even while I was still enjoying the part it did explain. That list of unexplained residue — the leftover symptoms a breakthrough hadn't touched — turned out to be the single most valuable thing the investigation produced, more valuable, in a strange way, than any individual discovery. It was the thing that kept pulling me forward every time a real success tried to convince me the work was done.

So here is the lesson, stated as plainly as I can state it, because I think it is the actual center of this book, more than any specific finding about an ankle or an adductor or a chain of muscles that wouldn't cooperate: a working explanation and a complete explanation are not the same thing, and the gap between them will not announce itself. Nothing about how correct an idea feels tells you how much of the problem it accounts for. The only way to find the edge of an explanation is to keep asking, deliberately and a little uncomfortably, what it still doesn't explain — especially, and this is the hard part, right after it has just explained something important.

That discipline does not end with a satisfying final answer. It ends, if it ends at all, with a model that has simply run out of things it obviously fails to explain — for now. That is a different kind of ending than the one most stories promise. It is, I have come to think, a more honest one.

Chapter Seven: What We Currently Believe

Here is where the model stands, as honestly as I can state it.

The left side of my body does not function as a single, coherent chain the way the right side does. It is, instead, offline in several distinguishable places at once — not one injury with many symptoms, but several separate, only partly related problems that happen to converge in the same general region and produce a single, unified-looking failure. That convergence is what made this so hard to solve for so long. A problem that looks like one thing is much easier to chase than a problem that actually is several things wearing one disguise.

I want to walk through what the current model actually contains, because the shape of it matters more than any individual item on the list. There is the original adductor injury — still relevant, still contributing some residual tightness, no longer treated as the explanation for anything beyond itself. There are two separate centers of restriction around the hip and glute, whose exact anatomical identity I still cannot name with confidence, but whose functional presence is no longer in any doubt. There is a foot and ankle region that releases, repeatedly, in ways that produce real but incomplete change. There is the ankle's muscular control — the discovery that reorganized the second half of this book — which explains a meaningful portion of the base-bracing and downward-pressure problem without explaining any of the proximal restrictions. There is a knee and thigh that still don't fully stabilize a step the way their counterparts do. There is a glute, pelvis, and trunk that still don't fully accept load on that side. And underneath all of it, there is the simple fact that even where the correct pattern is now accessible, it is not yet automatic — it still requires conscious, deliberate attention every time, rather than happening on its own.

I have deliberately just listed eight separate things. That number is not a failure of the investigation to find a single cause. It is the investigation's actual finding. For a long time, I wanted there to be one answer underneath all of this — one structure, one mechanism, one thing that, if I could just identify it correctly, would make the rest of the picture resolve on its own. I no longer believe that. Not because I stopped looking for it, but because I looked long enough, and carefully enough, to become confident it isn't there. The evidence does not support a single cause. It supports several genuine, partially independent causes that happen to share a body, a side, and a chain of connected tissue — which is exactly specific enough to look, from a distance, like one problem, and exactly separate enough to have required eight different investigations to actually understand.

I think this is the part of the model most worth explaining carefully, because it is easy to misread a list like this as a failure to simplify. It is the opposite. Simplifying it — collapsing eight distinguishable, separately evidenced problems into one tidy explanation — is exactly the mistake this entire book has been about avoiding. Every early chapter in this investigation was, in its own way, an attempt at exactly that kind of simplification: the adductor as the whole answer, release as the whole answer, one muscle or one joint or one discovery as the whole answer. Each simplification worked, briefly, and each one eventually ran out of room. The current model is not a simplification. It is what's left after every attempt at simplification failed for a specific, identifiable reason.

That is also why I have not thrown out a single one of the earlier explanations in this book, even the ones that turned out to be incomplete. The adductor explanation did not get replaced by the current model. It got absorbed into it, correctly scoped down to the one

thing it actually explains. The same is true of the proximal restriction findings, and the release findings, and the individual muscle-engagement findings, and the foot and arch findings. Nearly everything this investigation ever believed is still present in the current model, in a smaller, more accurately bounded form. Almost nothing was discarded outright. Most of it was simply cut down to the size the evidence actually supports.

There is one piece of this that is not a sub-issue in the same sense as the others, and I want to name it separately because I think it is, quietly, the most important item on the list. Even in the places where the correct movement pattern is now genuinely available — where the muscle fires, where the joint moves, where the chain, for a rep or a set, actually does what it's supposed to do — it does not yet happen without deliberate, conscious effort. I have to ask for it. It does not yet offer itself. That gap, between a pattern being *possible* and a pattern being *automatic*, is itself a distinct kind of unresolved problem, separate from every anatomical or mechanical piece on the list above it, and it is the piece the investigation is least equipped to resolve through observation alone. You cannot map your way to automaticity. You can only build toward it, through repetition, and wait.

That waiting is where the book ends. Not because the investigation is finished, but because this is genuinely where it stands.

Chapter Eight: Why the Investigation Continues

I do not have a tidy ending to give you, and by now I hope that isn't a disappointment so much as it is exactly what you were expecting.

An investigation that resolves itself completely, on schedule, with every question answered by the final chapter, is not a more honest investigation than this one. It's usually just a shorter one — stopped before its remaining questions had a chance to surface, or quietly edited, after the fact, to look more finished than it actually was. I have tried, throughout this book, not to do that. So I want to end by being specific about what is still unresolved, rather than gesturing vaguely at “more work to be done” and leaving it there.

I still do not know the exact anatomical identity of either of the two proximal restriction centers around the hip and glute, or whether they are related to each other, or whether either of them traces back to the original adductor injury this book opens with. I still do not fully understand the mechanism behind a specific set of neurological symptoms that show up during ordinary sitting — a genuinely separate thread this book hasn't had room to follow closely, but one the investigation has not forgotten about. I do not know exactly which muscles, or which specific actions, are responsible for each portion of the ankle's remarkable ability to organize engagement further up the chain, or whether that mapping will hold true across every kind of movement or just the ones I've tested so far. I do not know how much of the remaining foot tightness is still limiting the base, or whether continued improvement in ankle control will, on its own, start opening up hip motion that has been unavailable for years. And I do not know — this is the one I think about most — whether the integration I felt for the first time on August 28 will hold up on its own, or

whether it will need to be deliberately rebuilt, rep by rep, until it stops requiring deliberate rebuilding at all.

None of these are failures of the investigation. They are its current edges, and an investigation's edges are supposed to move. That has been true at every single point in this book. The questions at the end of Chapter Three were not the questions at the end of Chapter Four. The questions at the end of Chapter Six were not the questions I am sitting with now. Each answer this project has produced has done what good answers actually do — not end the questioning, but relocate it, sharpen it, point it somewhere more specific than where it started. I expect that to keep happening. I would be more worried if it stopped.

If there is a single thing I would want a reader to carry out of this book, it is not any specific finding about an ankle, or an adductor, or a chain of connective tissue that took years to understand. It is the shape of the process that got me there: notice something honestly, resist the urge to close the question before the evidence has earned that closure, keep what survives scrutiny and let go, carefully, of what doesn't — and treat every satisfying answer as a hypothesis about how far it reaches, not a proof that it reaches far enough.

I don't know yet how far this one reaches. I am still finding out. That, more than any conclusion I could offer you instead, is the actual, current, honest state of the investigation — and it is, I have come to believe, a perfectly good place for a book like this one to end.